TRAILS NEPAL

Altitude Sickness Guide

Quick reference guide for mountain trekkers

Golden Rule Never go higher with symptoms. If breathing is hard or the person is confused/unsteady, descend now.

(AMS)

Acute Mountain Sickness

SYMPTOMS

- Headache
- Rapid heartbeat
- Breathless
- Dizziness
- Insomnia
- Fatigue
- No appetite
- Nausea

PREVENTION

- Slow Ascend
- Stay hydrated (3-4L)
- Avoid alcohol
- Rest

HAPE

High-Altitude Pulmonary Edema

SYMPTOMS

- Chest congestion
- Rapid heartbeat
- Breathless
- Dry Cough
- Blue Lip
- Energy less
- Rapid breathing
- Breathless at rest

PREVENTION

- Ascend Gradually
- Avoid overexertion
- Take acclimatization
- Rest

HACE

High-Altitude Cerebral Edema

SYMPTOMS

- Severe Headache
- Drowsiness
- Confusion
- Loss of coordination
- Unsteady walking
- Altered behavior
- Stupor
- Coma in late disease

PREVENTION

- Ascend Gradually
- Dexamethasone for treatment
- Acetazolamide for prevention

Disclaimer: This guide is for informational and awareness purposes only and is not a substitute for professional medical advice, diagnosis, or treatment. Trails Nepal is not a medical organization.

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(AMS) Acute Mountain Sickness

WHAT IT IS?

Acute Mountain Sickness (AMS), often called High Altitude Sickness, occurs when your body doesn't have enough time to adjust to lower oxygen levels at higher elevations. It is a natural physiological response, but one that demands immediate attention and respect. Think of it as your body's signal to slow down and breathe.

PREVENTION TIPS

- ✓ Ascend slowly (no more than 300-500m per day above 3,000m)
- ✓ Stay hydrated (3-4 liters per day)
- ✓ Avoid alcohol and sleeping pills
- ✓ Eat light, high-carb meals
- ✓ Listen to your body and rest when needed

(HAPE) High-Altitude Pulmonary Edema

WHAT IT IS?

High-altitude pulmonary edema (HAPE) is a dangerous build-up of fluid in the lungs caused by altitude-related low oxygen, not by heart failure. It usually appears 24-96 hours after a rapid ascent, often above about 2,400 m, and it can occur with or without AMS. Early diagnosis matters because HAPE can progress quickly and may be more rapidly fatal than HACE.

PREVENTION TIPS

- ✓ Ascend gradually
- ✓ Once above 3,000 m, increase sleeping altitude by no more than about 500 m per night.
- ✓ Add an extra acclimatization night for every 1,000 m of sleeping-altitude gain.
- ✓ Avoid overexertion during the first 48 hours at altitude.
- ✓ Known HAPE-prone trekkers may need preventive medication from a clinician.

(HACE) High-Altitude Cerebral Edema

WHAT IT IS?

High-altitude cerebral edema (HACE) is a severe altitude illness in which the brain swells. It is generally considered the end-stage of AMS and is defined by neurologic symptoms, especially altered mental status and ataxia. It is rare, especially below about 4,300 m, but once it starts it can deteriorate rapidly to coma within hours if untreated.

PREVENTION TIPS

- ✓ Ascend gradually and avoid going too high too fast.
- ✓ Once above 3,000 m, limit sleeping-altitude gain to about 500 m per night.
- ✓ Add an acclimatization day for every 1,000 m of sleeping-altitude gain.
- ✓ Do not ascend with worsening AMS symptoms.
- ✓ Dexamethasone is used mainly for treatment or emergency backup, while acetazolamide is preferred for prevention of AMS during ascent.